

Family Planning and Five-year Plans

India became the first country in the world to adopt an official national population policy in support of family planning (Cadwell 1988, Visaria and Jain 1976). In the third five-year plan (1961-66), the objective of stabilising the growth of population within a reasonable period was put at the centre of India's planned development. The late 1960's saw a shift in the emphasis of family planning delivery from clinic-based approach to wider community extension strategy. In 1968 a demographic goal was set to reduce birth rate from 41 to 23 per 1000 within 10 years but the performance was far from desired.

By early 1970's India experienced very high rate of population growth. The 1971 Census showed a decadal increment of 109 million. Consequently, there was renewed emphasis on targets, compensation payments and male sterilisation. In the fifth five-year plan Prime Minister Indira Gandhi tried to implement population control using coercive and dictatorial powers. The Emergency in 1975-77 saw forced sterilisation in mass camps that ended up being a failure and undermining family planning in the country.

The rhetoric of the population control and forced sterilisation largely disappeared in the Sixth Plan (1980-85). However, family planning became more widespread in practice. Since Seventh Plan there were efforts to tailor family planning programme to the conditions prevailing in different states and adoption of multi-sectoral approach that recognised linkages between birth control and education programmes.

To achieve family planning goals government adopted following measures:

- 1) Motivation programme to spread knowledge of family planning through newspapers, radio, T.V., film, etc.
- 2) Involvement of private sector in contraceptive delivery to all rural and urban population.
- 3) Financial incentives and political restrictions to encourage family planning.
- 4) Extensive sterilisation of both males and females.

With the fall in fertility rate, there was preference for boys over girls that resulted in problem of missing women in India. In 2010, the ratio of males to females in India had reached 108 to 100, one of the highest in the world. Kerala emphasised on poverty reduction and human development and achieved a sharp decline in fertility rate in India. In Kerala, more than 85 per cent of women are literate, which means they have more power in household and opportunities in the work force. The success of Kerala suggested that by bringing in women empowerment, literacy, and human development we can bring in reduced fertility rates and preferences for small family could be improved.

3.4.2 National Population Policy, 2000

The National Population Policy, 2000 was announced on 15th February 2000 with an objective to control the rapidly growing population and stabilise it at reasonable level.

Immediate Objective

The immediate objective is to provide for facilities to meet the unmet needs for contraception, health care infrastructure and health personnel and an integrated service delivery for basic reproductive and child health care.

Medium-term Objective

The medium objective is to bring Total fertility rate (TFR) to replacement level by 2010.

Long-term Objective

The long-term objective is to stabilise population by 2045, at a level consistent with requirements of sustainable economic growth, social development, and environmental protection.

National Population Policy listed the following measures to achieve a stable population by 2046.

- 1) Reduction of infant mortality rate below 30 per 1000 live births.
- 2) Reduction of maternal mortality rate to below 100 per 1,00,000 live births.
- 3) Health insurance cover of Rs. 5000 for couples below poverty line, with two living children, who undergo sterilisation.
- 4) To achieve 80 per cent deliveries in regular dispensaries, hospitals, and medical institutions with trained staff.
- 5) A special reward for women who marry after 21 and opt for terminal method of contraception after the second child.
- 6) Self-help groups at village panchayat levels comprising mostly of housewives to engage with health care workers and gram panchayats.
- 7) Strict enforcement of Child Marriage Restraint Act and Pre-Natal Diagnostic Techniques Act.
- 8) Access to information containing AIDS, prevention and control of communicable diseases.
- 9) Incentive to adopt two child small family norms. The message of small family is to be spread through dissemination of information, education, and communication.
- 10) Facilities for safe abortions to be increased. Contraceptive technology and research in the reproductive and child health are to be encouraged.
- 11) Elementary education to be made free and compulsory.
- 12) A National Commission on Population headed by Prime Minister to monitor implementation of new policy.
- 13) A National Population Stabilisation Fund renamed as Janasankhya Sthirata Kosh (JSK) to support projects schemes, initiatives and innovative ideas designed to help population stabilisation and provide window for canalising resources through voluntary contributions.

Achievements

It has been estimated that 320 million births have been averted during the period 1956-2011 through family welfare programme. The incidence of acceptance of family planning methods peaked at 62.9 million at the beginning of 2011. The couple protection rate has gone up to 48.0 per cent (against the world average of 61 per cent).

It brings out that the demographic transition has already set in India and is moving swiftly to its final stage. The resulting differential impact on fertility level and on population growth rate is clearly reflected in 2011 census. India will reach the threshold of the Net Reproduction Rate of 1 within a decade from now. The desired family size is already close to replacement level in 10 states. In other states it is much lower than the actual number of children born.

The population policy seems to have great potential for attaining the goal of a stable population in the stipulated time frame. There are opportunities that are untapped. If we seize these opportunities we can reduce birth rate, infant mortality rate and promote adoption of small family norms in the country.

Limitations

- 1) Population control programme is led by government bureaucracy, which can be effective in addressing technical matters, but here we need transformation in the attitudes of people in the favour of small family norm.
- 2) The main targets laid down in the Population policy are over-ambitious. Goals like making school education up to age of 14 free and compulsory, reducing school drop-out rates to below 20 per cent for both girls and boys, achieving TFR of 2.1 by 2010, reducing infant mortality rate to 42 per 1000 live births, reducing maternal mortality rate to below 100 (from 437) per 100000 live births, achieving universal immunisation of children against all vaccine preventable diseases are too ambitious to be achieved in the stipulated timeframe.
- 3) The incentives mostly in cash resulted in widespread corruption and cooking of data. These incentives did not reach people who were living in areas where the fertility is high (urban slums, tribal communities). The social transformation can only be brought with effective implementation of policies at the grass root level and requires an active participation of people.

Check Your Progress 2

- 1) Critically examine the link between population growth and economic development in India.

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